



MedCare Radiology & Imaging Department IMAGING RESULTS REPORT

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Reg No: MH/RAD/2015/00789 | NABH Accredited | ACR Certified

Report No: IMG-2024-00456

Report Date: 12 January 2024

Radiologist: Dr. Rajesh Gupta (MD – Radiology)

FINAL REPORT

PATIENT INFORMATION

Name: Rahul Sharma

Patient ID: PT-2024-00123

DOB: 15 May 1990 | Age: 33 Yrs

Gender: Male | Blood Group: B+

Allergies: Contrast dye – None reported

Weight: 72 kg | Height: 175 cm

Phone: +91-9876543210

REFERRAL & CLINICAL INFORMATION

Referred By: Dr. Anil Kapoor (MS – Ortho)

Clinical Indication: RTA – Multiple Trauma Assessment

Date of Imaging: 12 January 2024, 09:15 AM

Modalities Used: X-Ray, CT Scan, Ultrasound, MRI

Contrast Used: IV Contrast (Iohexol 300mg/mL) – CT

Accession No: ACC-2024-0089-MCH

Priority: **STAT – Emergency**

IMAGING SUMMARY

9

Total Studies

5

Abnormal

3

Normal

1

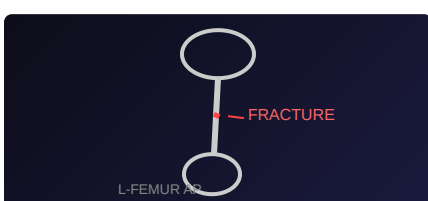
Pending

4

Critical Findings

IMAGING STUDIES OVERVIEW

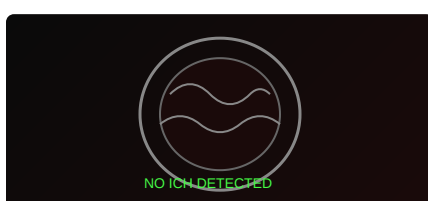
X-Ray – Left Femur (AP/Lateral)



Study: X-Ray | Region: Left Femur

Finding: **Mid-shaft Transverse Fracture**Result: **Abnormal**

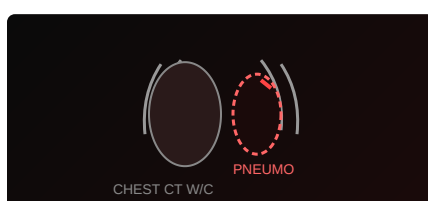
CT Scan – Head (Non-Contrast)



Study: CT Scan | Region: Brain / Head

Finding: **No Intracranial Hemorrhage**Result: **Normal**

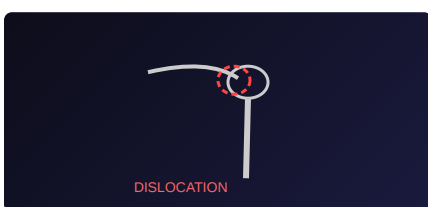
CT Scan – Chest (With Contrast)



Study: CT Chest | Region: Thorax

Finding: **Right Pneumothorax + Rib Fracture**Result: **Abnormal**

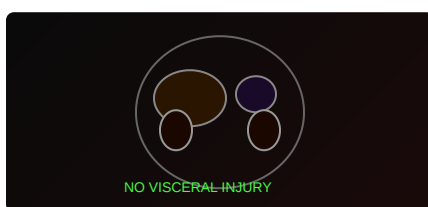
X-Ray – Right Shoulder (AP)



Study: X-Ray | Region: Right Shoulder

Finding: **Anterior Glenohumeral Dislocation**Result: **Abnormal**

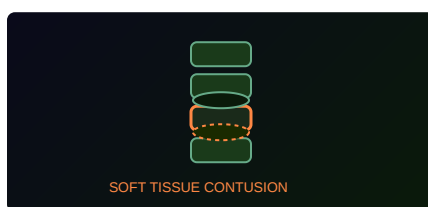
CT Scan – Abdomen & Pelvis



Study: CT Abdomen | Region: Abdomen/Pelvis

Finding: **No Solid Organ Injury**Result: **Normal**

MRI – Lumbar Spine (L3-L4)



Study: MRI | Region: Lumbar Spine

Finding: **L3-L4 Soft Tissue Contusion**Result: **Abnormal**

DETAILED IMAGING FINDINGS

#	Study / Modality	Body Region	Date & Time	Key Findings	Impression	Priority	Result
1	X-Ray (AP + Lateral)	Left Femur	12-Jan 09:15	Mid-shaft transverse fracture with 3mm displacement. Cortical breach noted. No comminution.	Closed Femur Fracture – ORIF indicated	STAT	Abnormal
2	CT Scan (Non-Contrast)	Brain / Head	12-Jan 09:30	No intracranial hemorrhage. No midline shift. Mild cerebral edema noted. Skull intact.	Mild TBI / Concussion – No surgical intervention needed	STAT	Normal
3	CT Scan (With IV Contrast)	Chest / Thorax	12-Jan 09:35	Right-sided pneumothorax (~25%). Fracture of 5th & 6th ribs. No hemothorax. Left lung clear.	Pneumothorax – Chest tube insertion required	STAT	Abnormal
4	X-Ray (AP View)	Right Shoulder	12-Jan 09:20	Anterior subcoracoid dislocation of right glenohumeral joint. No associated fracture.	Shoulder Dislocation – Closed reduction indicated	Urgent	Abnormal
5	CT Scan (With Contrast)	Abdomen & Pelvis	12-Jan 09:40	No solid organ injury. Liver, spleen, kidneys intact. No free fluid in abdomen. Pelvis intact.	No Abdominal Injury – Conservative management	Urgent	Normal
6	X-Ray (AP + Lateral)	Chest (Ribs)	12-Jan 09:18	Fracture of right 5th and 6th ribs. No sternal fracture. Mild pleural effusion on right.	Multiple Rib Fractures – Pain management + monitoring	Urgent	Abnormal
7	Ultrasound (FAST Exam)	Abdomen	12-Jan 09:10	No free peritoneal fluid. No pericardial effusion. Bladder intact. Bowel loops normal.	Negative FAST – No internal bleeding	STAT	Normal
8	MRI (T1 + T2 Weighted)	Lumbar Spine L3-L4	12-Jan 11:00	Soft tissue contusion at L3-L4 level. Mild disc bulge at L4-L5. No spinal cord compression. No fracture.	Soft Tissue Injury – Physiotherapy recommended	Routine	Abnormal
9	MRI Brain (With Contrast)	Brain	Pending	Ordered for detailed TBI follow-up assessment. To be done on 25-Jan-2024.	Awaiting – Follow-up Study	Routine	Pending

Total Imaging Studies: 9 Studies

KEY MEASUREMENTS & CRITICAL VALUES

Fracture Measurements

Left Femur Fracture Displacement	3.2 mm
Fracture Type	Transverse – Mid-shaft
Bone Alignment	Mild Angulation – 8°
5th Rib Fracture – Gap Width	2.1 mm
6th Rib Fracture – Type	Non-displaced, Hairline

Lung / Thorax Measurements

Pneumothorax Size (Right)	~25% Collapse
Pleural Effusion (Right)	Minimal – ~50 mL
Left Lung – Status	Clear – No Pathology
Heart Size (CTR)	0.48 (Normal)
Mediastinum	Midline – No Shift

Neurological Measurements

Cerebral Edema	Mild – Diffuse
Midline Shift	None Detected
Ventricle Size	Normal
GCS at Imaging	12 / 15
Skull Integrity	Intact – No Fracture

Spine & Soft Tissue

Lumbar Region Affected	L3-L4 (Contusion)
Disc Bulge	L4-L5 – Mild
Spinal Canal Compromise	None
Shoulder Joint Space	Widened – Dislocated
Soft Tissue Swelling	Moderate – L3-L4 Region

RADIOLOGIST'S FINAL IMPRESSION & RECOMMENDATIONS

Overall Impression: Imaging confirms multiple traumatic injuries consistent with high-velocity road traffic accident. Most critical findings are the left femur mid-shaft fracture requiring surgical fixation (ORIF) and right-sided pneumothorax (~25%) requiring immediate chest tube decompression.

Neurological: CT head is reassuring with no evidence of intracranial hemorrhage or midline shift. Mild diffuse cerebral edema consistent with concussion. Clinical correlation and GCS monitoring advised. Follow-up MRI brain recommended on 25-Jan-2024 for complete TBI assessment.

Thoracic: Right pneumothorax and rib fractures (5th & 6th) confirmed. Pleural effusion is minimal and non-significant. Post-tube chest X-ray advised to confirm lung re-expansion. Serial chest X-rays every 24 hours recommended.

Musculoskeletal: Anterior shoulder dislocation confirmed — successful reduction documented on post-reduction X-ray. Lumbar soft tissue contusion at L3-L4 — no neurological compromise. Physiotherapy recommended after acute phase.

Abdominal: FAST exam and CT abdomen are negative for solid organ injury and intraperitoneal free fluid. No surgical intervention required for abdomen.

Follow-up Imaging Recommended: (1) Post-op X-Ray femur after ORIF, (2) Chest X-Ray 24h post chest-tube, (3) MRI Brain – 25 January 2024, (4) X-Ray femur at 6-week follow-up to assess healing.